Recommended, reimbursed — and still not completed.

A partnership brief on closing the gap between the preventive care that is recommended and the preventive care that actually gets finished — in the safety-net settings where the gap is widest.

THE PITCH IN 30 SECONDS

PROPOSED · IN DEVELOPMENT

PROBLEM	U.S. colorectal-cancer screening sits at **63.5% against a 72.8% target** — and 44.89% in Federally Qualified Health Centers (CY2025 UDS; Michigan 48.74%). The services work; **completion** is what fails.
SOLUTION	A proposed **weekly routine**, run by the site's own staff: find who is overdue, rank by three transparent rules, remove friction, and follow every positive test to its next step, tracked on a weekly dashboard. A predictive model is on the roadmap — not in the first pilot.
EVIDENCE	Every lever is already recommended by the Community Preventive Services Task Force (mailed FIT +16.1pp, reminders +15.3pp, navigation +13.6pp). Honest caveat: real-world gains depend on **how well each step is carried out** — STOP CRC averaged ~3.4pp — which this design exists to protect.
COST	No fee to the site for the first three months. Published benchmark: **~$90** per additional person screened by mailed-FIT outreach (Pignone et al., JGIM 2021; total cost net of background screening, 2019 dollars; $54.83 direct per person screened in the same study) — not the pilot's cost, which the feasibility phase measures.
THE ASK	One clinic to run a **12-month pilot**: a **3-month feasibility phase at no fee**, then a funded pilot to month 12 if it works. Step one is a 30-minute conversation.

THE PROBLEM — ADOPTION, NOT SCIENCE

~8%
of U.S. adults 35+ receive all 15 high-priority preventive services (Borsky, *Health Affairs* 2018).

63.5% → 72.8%
CRC screening vs. the Healthy People 2030 target — barely off baseline (2023).

44.89%
CRC screening in Federally Qualified Health Centers, CY2025 UDS (Michigan 48.74%) — the gap concentrates here.

WHICH ONE ARE YOU? — DIFFERENT PARTNERS, DIFFERENT VALUE, SAME ENGINE

HEALTH CENTERS / FQHCS

Improve a measure you already report

Host the pilot — raise a UDS/HEDIS measure you already report, inside your existing staffing. The first three months carry no fee, and you decide at month three whether to go on.

FUNDERS / PARTNERS

Fund a cost-effective, scalable model

Fund months 4–12 once the feasibility phase shows the routine can run. Equity-stratified design, criteria agreed before results, and a path from one site to a network.

RESEARCHERS

Co-design a publishable pilot

An independent evaluation: before and after against the site's own trend, with a comparison clinic where possible, reported with RE-AIM. Plan published first; authorship by contribution. Principal investigator: to be confirmed.

THE PILOT LOOP — THREE CORE PILLARS, AND A THREE-PILLAR ROADMAP

CORE · WHAT THE FIRST SITE RUNS

- Service redesign & friction reduction
- Interoperable data infrastructure — one nightly coded 19-column file for the pilot, dates as day counts, key kept at the site
- Implementation enablement — training so the site runs and sustains it

ROADMAP · AFTER A VALIDATED PILOT, NOT IN THE FIRST PILOT

- Predictive AI, explainable
- Agentic & conversational outreach
- Algorithmic equity & governance

Identify → Rank by transparent rules (days overdue, days since a positive test, prior no-shows) → Reduce friction → Communicate (literacy-tailored) → Escalate (community-health-worker outreach) → Measure, incl. diagnostic follow-up after a positive result. No patient messaging is automated in the first pilot.

THE DEMONSTRATION

A proposed 12-month colorectal-cancer completion pilot — the first three months at no fee.

One Southeast Michigan health center below the UDS benchmark, with outreach staff and a quality-improvement lead. **Months 1–3: feasibility, no fee** — kits sent and returned, positive tests navigated and booked, days to a booked colonoscopy, staff hours, data completeness. **Month 3: decision** — the site funds the rest, a funder is found, or it stops. **Months 4–12: funded pilot.** The site keeps its workflow and its data; HSREP is implementation lead (about 30 hours a week); an academic partner evaluates independently, before and after against the site's own trend with a comparison clinic where possible and a target published in advance. First readout about month six; results published whichever way they go.

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An HSREP initiative in development. All designs are illustrative and future-tense; no patient data has been collected. The first pilot runs on transparent rules and existing staff; AI components are a roadmap item. Operational materials will be reviewed by counsel before any launch.

SOURCES — Borsky et al., Health Affairs 2018 · Healthy People 2030 C-07 (2023 baseline) · HRSA Uniform Data System CY2025, national and Michigan, accessed Aug 2026 · CDC Community Guide (median effects) · Coronado et al., STOP CRC, JAMA Internal Medicine 2018 · Pignone et al., JGIM 2021 (mailed-FIT cost).